Lunch card. Thursday 2026-11-05, El Paso. Luis, Fernando, Jesse.

The five lines to land

1. **Four in five appeals win. Almost nobody files one.** 11.5% of denied prior auths appealed, 80.7% overturned. Medicare Advantage, all services, not an imaging number; the only audited national figure there is.
2. **13 hours a week, about 40 requests, and 40% have someone working only on this. You have not hired that person yet.** AMA 2025 survey, self-reported.
3. **CMS priced the same burden at 220 million provider hours** over ten years, in the rule itself. A projection, not measured savings. Then: 95% of MRI prior auths are approved. A process that approves 95% of what it touches is one you automate, not one you staff.
4. **22.3% cancelled versus 2.0% no-show, seventy percent with notice.** Backfill, not reminders. UC Irvine, one center; your numbers will differ, the pattern will not.
5. **5.1% to 3.8% with a text reminder. 1.3 points, not "25%."** Same study. Your RIS will do this. Let it.

The one-sentence close

"Because the decisions that are free today stop being free the day you sign your first vendor contract, and finding out if I'm right costs you one phase, not the whole engagement."

If short on time, skip in this order

17 (owner numbers), then **24** (keep its question for the Thank You), then **14** to one sentence, then **8 and 9** to their closing lines, then **19** to two sentences. Never skip 3, 4, 6, 11, 18, 19, 22, 23, 25 or 26. At 31:00 and not yet on 22: jump to **22, then 25, then 26.** The price lands at about 35:30; the Thank You at 41:00.

Never say

- Luis is your RadAssist partner, or any arrangement of Luis's. "Luis uses software I built. Anything between us on that is separate and in writing."
- Building, site, address, property, or what was there before. Only "your new center."
- Scale, platform, roll-up, Akumin, the lawsuit, Desert Imaging, "when you sold to private equity." If he raises any of it: listen, say little, move on.
- A vendor winner, score or ranking, or any vendor price. "Every price in this market is a quote; the pick is Phase 2."
- Any three-year total. Phase 3 produces it. Running costs are "about two thousand a month at expected volume, an estimate, with a cap."
- Headcount reduction or layoffs (it "changes what your people do"); "Jesus"; "you open in June" as a fact.

The demo in five lines (slide 11)

1. Tabs already open on the phone (cellular) and the laptop; banner checked: green is live, orange is simulated, say which.
2. **Maria:** order, needs auth, packet, approve and submit, advance 3 days, approved.
3. **Robert:** same steps fast, advance twice, denied, review appeal, sign and submit, advance, overturned. Then the Owner's Monday report.
4. Say it before anyone asks: the test server has no medical-necessity rules, so the approve or deny is simulated. Everyone waits for you on **Advance 3 days.**
5. Reload the page to reset (Cmd+R, or pull down on the phone); Start over does not. Reload before lunch and before handing the phone over.

The twelve answers, one line each

1. "**You're one guy. Why trust you?**" Luis uses my software on real patients every day, you'll click a working demo in a few minutes, and every phase is a fixed fee you sign off, so if I don't deliver Phase 1 you've spent seven thousand, not forty-five.
2. "**Hit by a bus. Who fixes it at 7 a.m.?**" Scheduling runs in the system you buy, with that vendor's support line; if my piece goes down your staff do it by hand; and you own the code, the specification and a runbook that says who to call.
3. "**$45k before a single patient. Why now?**" You're spending seven for Phase 1, then you decide; the data terms are only free before you sign a RIS or PACS vendor, and fixing them later means renegotiating or rebuilding.
4. "**If AI writes the code, why not a kid from UTEP?**" Anyone can buy the scanner; you pay for the person who knows what the image means: what the center needs on day one, which terms protect your data, what Texas law requires, and when the AI is confidently wrong.
5. **Jesse:** "**Where does patient data live, who sees it, the AI company?**" In the United States, always; by role, every view logged, your own isolated instance with its own database and keys; the AI company only under a signed business associate agreement, never kept, never trained on; a human signs anything clinical.
6. "**What if in March it takes longer and costs more?**" The price is fixed: fourteen stays fourteen; if the date moves the timeline moves, not the price; anything new is quoted and approved in writing before I touch it.
7. "**Why fixed fee? Did you pad it?**" With hourly every surprise is your cost, with fixed it's mine; each phase is priced separately with its deliverables listed, so you see exactly what seven buys, and you can walk after it.
8. "**Luis is your neighbor. How do we keep it clean?**" On paper: a written contract, fixed scope, a price per phase, your sign-off before each one; Luis isn't the referee, you and I deal directly, and you can stop after any phase. Never "RadAssist partner."
9. "**What do you need, and what do we get from Phase 1?**" A few hours with all three of you (Jesse, especially on the technical and integration decisions), your services, volumes and payer mix, and the opening date; six weeks later, five documents you own: requirements, the patient-flow map, the contract terms, a make-or-buy for each requirement with a running cost for anything built, and the shortlist with criteria. Four at signing, three on delivery.
10. "**What will it cost to run?**" Not in the forty-five: accounts in your name, at cost, no markup; a ballpark in Phase 1 and three hosting options with a total cost of ownership in Phase 3; around two thousand a month at expected volume, with a cap I can't pass without your OK.
11. "**What happens after opening?**" A support plan, a flat monthly fee for a set number of hours so you always know who to call and what it costs; or a role on your team, which we can talk about. (Three thousand a month for up to twenty hours, one-fifty beyond, from June, twelve months, ninety-day check-in.)
12. "**One sentence. Why say yes today?**" Because the decisions that are free today stop being free the day you sign your first vendor contract, and finding out if I'm right costs you one phase, not the whole engagement.

Three more, in one breath. "Can you guarantee the opening date?" No one honestly can; every date is conditional on the assumptions slide; what I guarantee is the price. "What if Luis leaves?" The contract is with the center; you own everything I build; nothing depends on one person, including me. "Who do you contract with?" I'll contract with whichever company runs operations, and the practice's patient data stays the practice's.

Scorecard (slide 18): "Eight candidates. I'd build five, buy two, and integrate the voice system." "The right-hand column is what each one actually does. Take a second with it." Quiet 15 to 20 seconds. Then the five builds, one line each. Close: "Any of these you'd push back on?" Stop talking.

Owner numbers (slide 17), two views of one system: owners get operations, uptime and costs against budget; the practice gets collections, denials, revenue by payer. If the split draws a question: "You run on fixed fees, so your numbers are costs, uptime and flow. Dr. Ramos's practice sees the money it collects. Same system, two views, the line your structure draws."

Ask Luis today, before the lunch: (a) who signs for the operating company, and does the December 18 capital date affect when the $4,000 retainer can be paid; (b) "Dr. Luis Ramos" or "Luis Ramos-Duran, M.D." on the cover, and "J. Fernando Escarzaga"?; (c) is "Quantum Radiology" the public name to use; (d) does the practice bill both the technical side (scanner and technologist) and the professional side (the read) of each scan, so every prior-auth denial lands on the practice?